

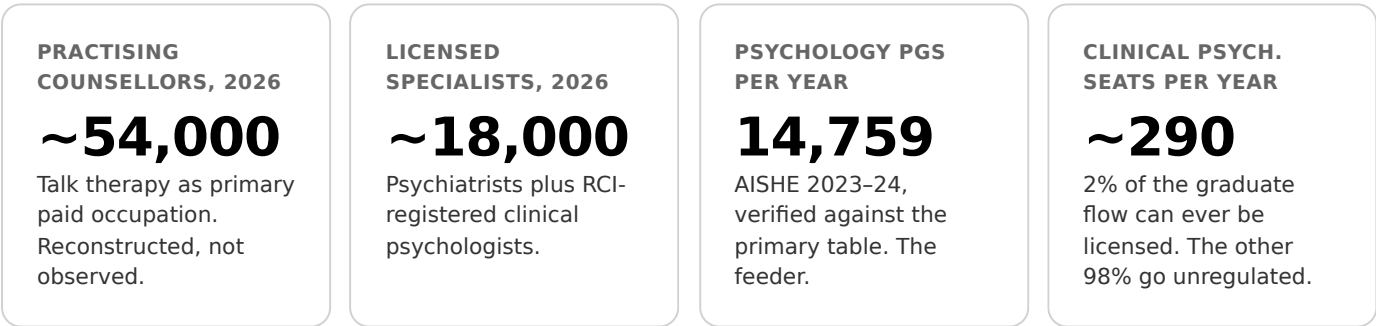
How many people in India actually do counselling for a living?

Nobody knows. There is no register, no census and no reliable estimate. This reconstructs the number from the one hard flow figure that exists, benchmarks it against countries that do count, and projects it to 2036 under three scenarios.

12 August 2026 · Base year 2026 · horizon 2036 · Prepared for the Aman market section

Every number you have read about India's mental health workforce is either a decade old, a single journal letter repeated until it sounded official, or a consultancy figure measuring psychiatric pharmaceuticals. The counselling layer — the people who would actually use practice software — has never been counted at all.

That is not a gap you can close by searching harder; it is structural. “Counsellor” is not a protected title in India. The Rehabilitation Council of India Act 1992 names *Clinical Psychologist* as one of seventeen registrable categories and does not name counselling psychologist at all, so there is no register to count against. What follows builds the number from the supply pipeline instead, then stress-tests it against international ratios.



The figures in circulation are worse than you think

“0.75 psychiatrists per 100,000” is the most-quoted statistic about Indian mental health. It comes from a single one-page letter in the *Indian Journal of Psychiatry* in 2019, whose authors triangulated “about 9,000 and counting” psychiatrists from a professional-society directory, a human-rights-commission report and a pharmaceutical company's conference attendance data, then divided by population. The Ministry of Health's own February 2025 press release attributes the figure to “the Indian Journal of Psychiatry” rather than to any government count. India's last official submission to the WHO Mental Health Atlas, in 2016, said 0.29.

India has no WHO Mental Health Atlas country profile for either the 2020 or the 2024 round.

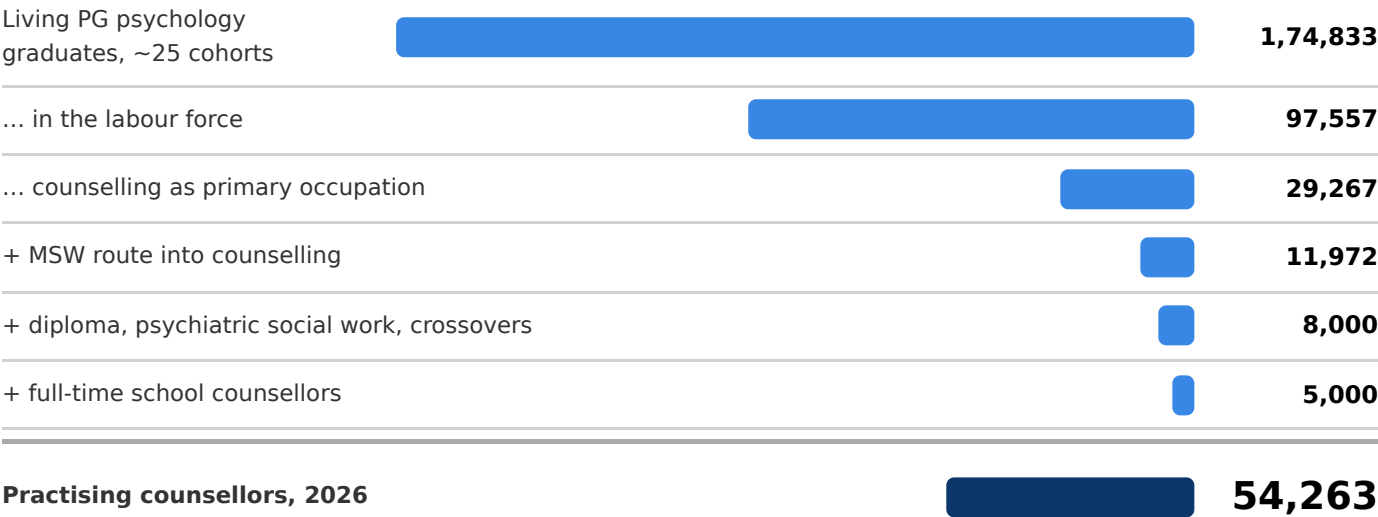
India responded to the 2024 questionnaire — it is listed as a contributing country — but no India-specific profile was published. Any current-sounding figure claiming a WHO source is repeating 2016 data, or repeating the 2019 letter under a WHO label it does not carry.

The second problem is that the stale figure is now materially wrong. Psychiatry PG seats have grown roughly fivefold since 2010 (266 seats across 112 institutions then; about 1,450 MD, DNB and DPM places now). Rolling the 2019 anchor forward through actual seat growth, a three-year training lag, 90% completion and 3% annual attrition puts India at roughly **13,000 psychiatrists in 2026**, not 9,000. The public conversation is running on a number that was already an estimate and is now seven years out of date.

A third inconsistency is more revealing. RCI's clinical psychologist register went from 2,840 in 2023 to 4,309 in July 2025 — roughly 765 additions a year. But the M.Phil Clinical Psychology programmes that feed it are widely reported to offer only about 290 seats. The register is growing at 2.6× its stated intake, which means either the published seat count is a serious undercount, or a large registration backlog is clearing. Both are worth knowing; neither is in any published source.

One hard number anchors everything else

The All India Survey on Higher Education publishes discipline-level postgraduate out-turn. For 2023–24 it records **14,759 M.A./M.Sc. Psychology graduates** (3,557 men, 11,202 women) against 54,484 enrolled. This is a real government table, verified directly against the primary PDF, and it is the only hard flow figure in the entire sector. Note what it implies: with roughly 290 clinical psychology seats a year, about **2% of psychology postgraduates can ever become licensed clinicians**. The remaining 98% who want clinical work have nowhere to go but unregulated counselling. India's informal layer is not a regulatory failure at the edges — it is the main channel.

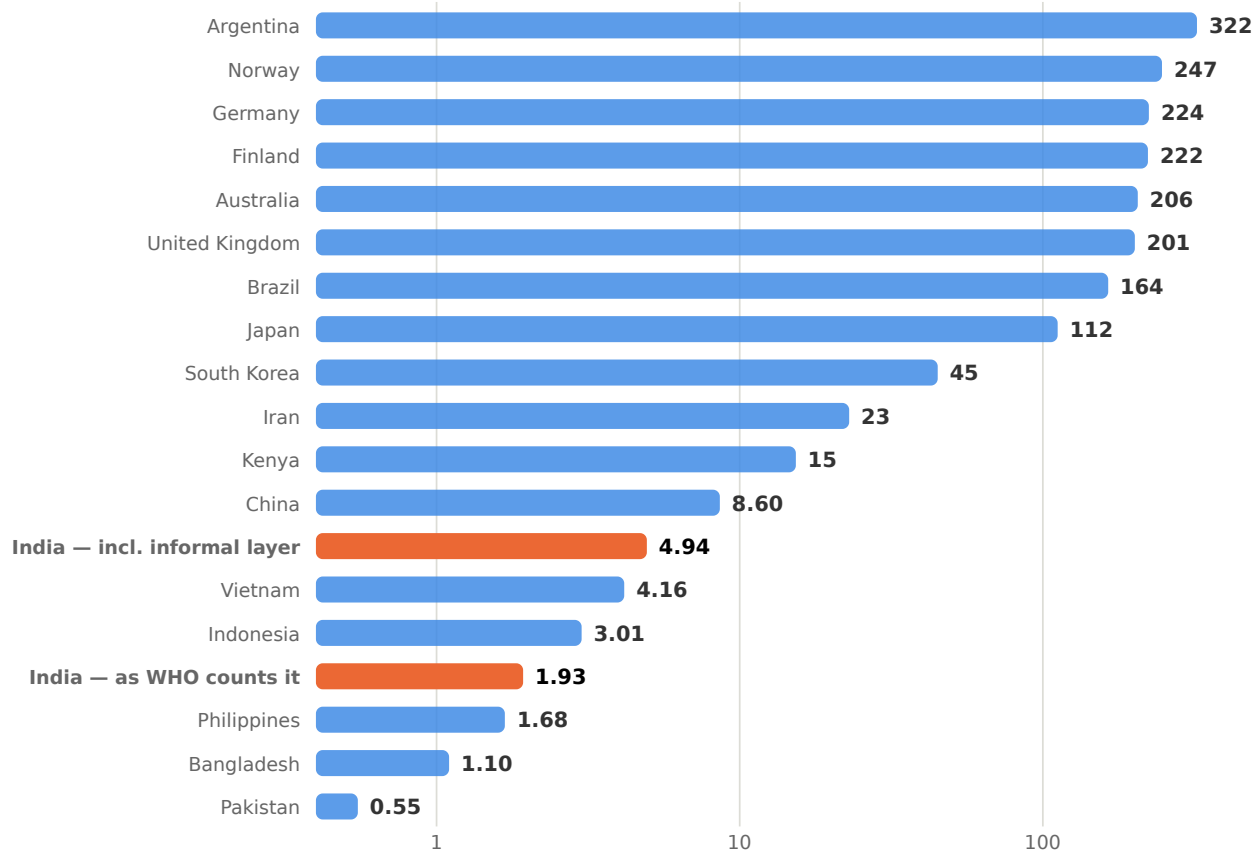


Why not just apply the international ratio? In countries that count both, counsellors outnumber psychiatrists by 6× to 20×. Applied to India's ~13,000 psychiatrists that would give 78,000–260,000. This model lands at 4.2×, below the band — deliberately. Ratio transfer assumes India's counselling layer is as economically viable as the counted countries'. A 2026 survey of Indian therapists found a majority earning ₹3 lakh a year or less while carrying 10–15 clients a week. A market that cannot pay its practitioners a living wage does not sustain a US-sized counsellor-to-psychiatrist ratio.

India is not merely behind rich countries. It is behind its own peer group.

Mental health workers per 100,000 population

WHO Mental Health Atlas 2020 country profiles, except India. Log scale — the spread is three orders of magnitude.

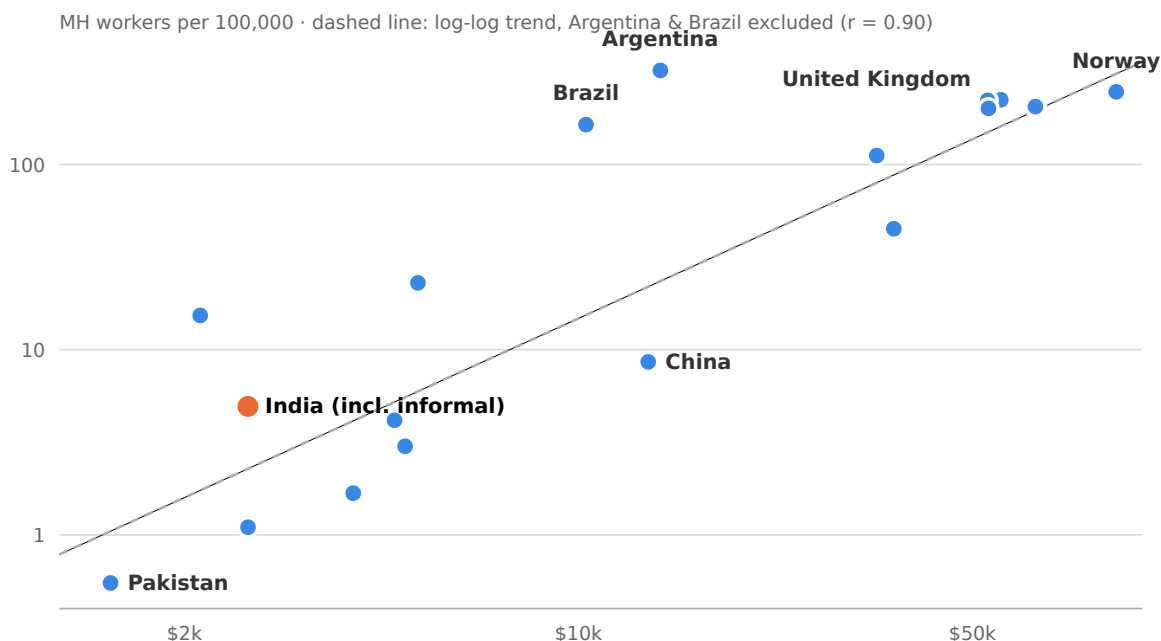


India appears twice: as its own government last reported to WHO in 2016 (1.93), and as this model counts it once the unregulated counselling layer is included (4.94). Even the generous reading leaves India below China and roughly 40× below the UK; the official reading puts it below Indonesia and the Philippines. The USA and Nigeria reported no workforce data to WHO at all and are excluded.

The income relationship is real but not deterministic. Across the countries above, log GDP per capita correlates with workforce density at $r = 0.72$, rising to 0.90 once Argentina, Brazil and Israel are excluded as cultural outliers. Argentina, at roughly India's income five times over but nothing like a rich country, has 286 psychologists per 100,000 — the highest density on earth, a function of a psychoanalytic culture rather than of money. Policy and culture can override income. That is the whole case for the optimistic scenario below.

Income explains most of it — but not all of it

GDP per capita (2024 US\$) against total mental health workers per 100,000. Both axes logarithmic.



Countries above the trend spend more attention on mental health than their income predicts; countries below spend less. India sits below trend even on the generous count — there is room to move that does not require India to get richer first.

The cautionary tale nobody in Indian policy discussion cites

China ran the experiment India is drifting toward. From 2002 its labour ministry issued a national psychological counsellor certification. By 2016 roughly **897,000 people held it** — about 180× the number of accredited clinical psychologists. Only 30–40% ever did any counselling work, and most of those part-time or as a hobby. In 2017 the government abolished the exam outright, having concluded the credential had become window dressing. On paper China had a 20× counsellor-to-psychiatrist ratio; in practice roughly 1×.

The lesson is not that certification is bad. It is that *credential counts are not capacity*. India's Budget pledge to train 100,000 allied health professionals including mental health counsellors, and NIMHANS Digital Academy's cumulative trainee figures (reported as both 42,000 and 176,454 by two credible government-adjacent sources, an unreconciled discrepancy), measure the same thing China was measuring. A projection that counts them as practitioners would be wrong by an order of magnitude.

Three scenarios, separated by whether anyone pays

The scenarios do not differ mainly on training capacity. They differ on whether a *payer* appears. India already produces far more psychology postgraduates than the market absorbs into practice; the binding constraint is that a counsellor cannot reliably earn a living, so entrants churn out. Every historical episode of fast workforce growth — UK IAPT from 2008, Australia's Better Access from 2006, the US post-parity decade — was triggered by a payer, not by a training expansion.

SCENARIO A

Credential Drift

The NCAHP behavioural-health council never builds a working counsellor pathway. Insurance parity stays unenforced. Practice economics stay poor, so churn stays high and the credentialed pool grows faster than practice does.

- Psychiatry seats +5%/yr
- Graduate flow +3%/yr
- Practice conversion falls 30% → 26%
- Annual exit from practice 9%
- CBSE school mandate goes unenforced

SCENARIO B · BASE

Steady Formalisation

NCAHP registers counsellors over three to five years. Corporate EAP demand keeps compounding from a real base. The platform sector consolidates and grows. The CBSE counsellor mandate is partially enforced.

- Psychiatry seats +8%/yr
- Graduate flow +5%/yr
- Practice conversion rises 30% → 34%
- Annual exit 7%
- 22,000 school posts absorbed by 2036

SCENARIO C

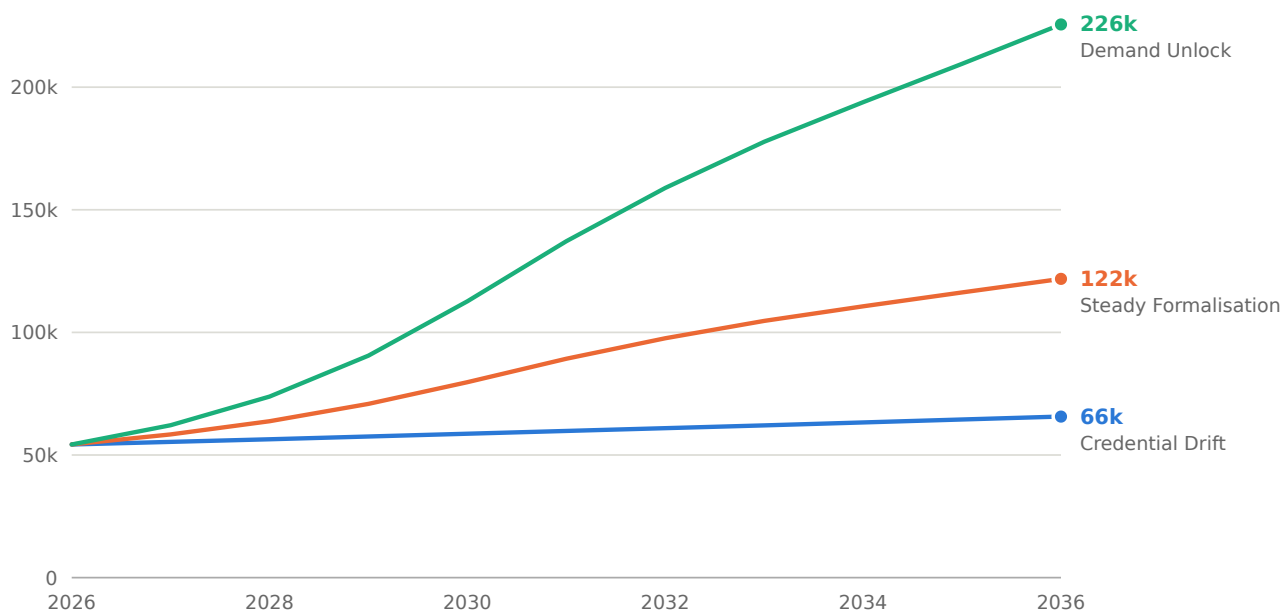
Demand Unlock

A payer event fires: IRDAI outpatient parity actually enforced, or a Better Access-style subsidised session scheme, or Tele-MANAS converted from triage into a delivery channel that pays panel counsellors.

- Psychiatry seats +11%/yr
- Graduate flow +7%/yr
- Practice conversion rises 30% → 46%
- Annual exit falls to 5%
- 62,000 school posts absorbed

Practising counsellors, 2026-2036

People whose primary paid occupation is talk therapy or counselling.



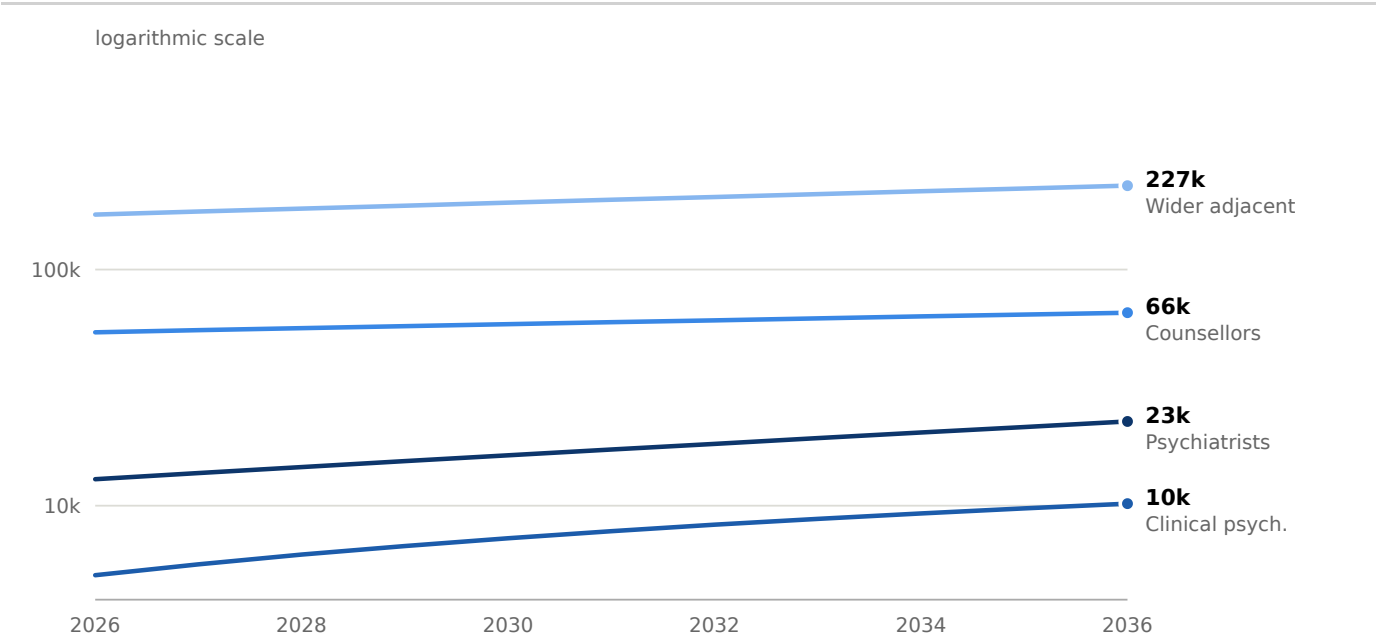
The fan is wide because the parameter that matters most — what share of psychology graduates can sustain counselling as a primary job — is unmeasured in India and modelled from China's documented 30–40% practice rate. By 2036 the scenarios differ by 3.4x.

Workforce composition, all three scenarios

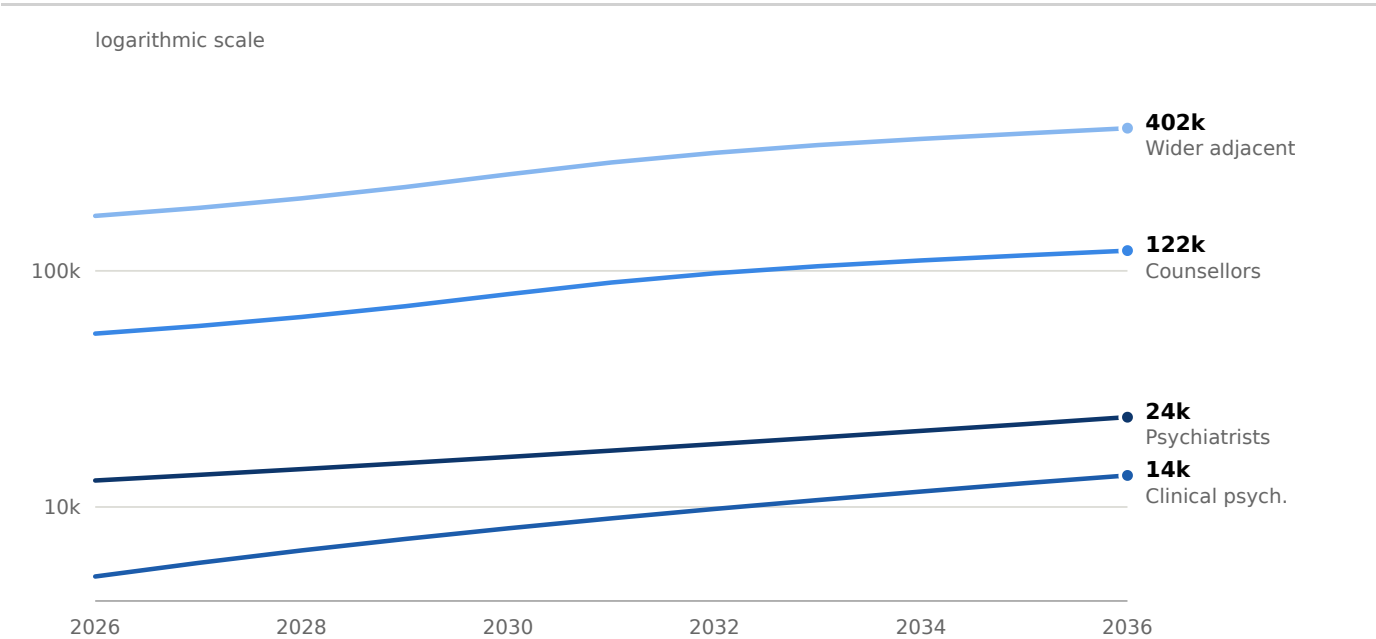
All four practitioner tiers on a logarithmic scale — they span two orders of magnitude, so a linear axis buries the licensed tiers entirely. The tiers are nested, not additive: every counsellor is inside the wider layer. Note that under Credential Drift the counsellor line flattens while the licensed lines keep climbing, so the gap between regulated and unregulated practice actually *narrows*; under Demand Unlock it widens sharply.

■ Psychiatrists ■ RCI clinical psychologists ■ Counsellors (primary occupation) ■ Wider counselling-adjacent

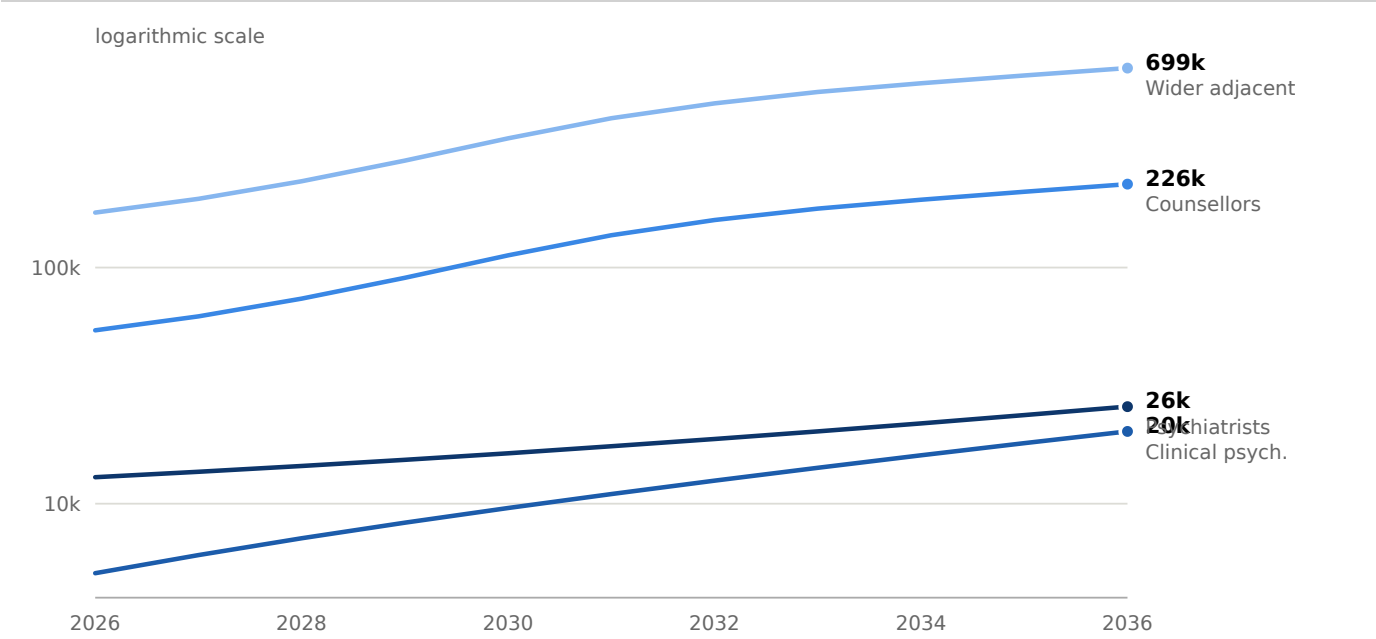
Credential Drift



Steady Formalisation



Demand Unlock



The full numbers, all three scenarios

Credential Drift

YEAR	PSYCHIATRISTS	CLINICAL PSYCH.	COUNSELLORS	WIDER LAYER	PSYCH./LAKH	ALL/LAKH	C : P RATIO
2026	12,942	5,074	54,263	1,70,928	0.89	4.94	4.2×
2027	13,738	5,647	55,350	1,76,012	0.93	5.07	4.0×
2028	14,569	6,206	56,447	1,81,196	0.98	5.21	3.9×
2029	15,436	6,751	57,556	1,86,481	1.03	5.34	3.7×
2030	16,344	7,282	58,676	1,91,872	1.09	5.48	3.6×
2031	17,292	7,800	59,809	1,97,370	1.14	5.61	3.5×
2032	18,284	8,305	60,954	2,02,977	1.20	5.76	3.3×
2033	19,322	8,797	62,112	2,08,695	1.26	5.90	3.2×
2034	20,408	9,278	63,282	2,14,527	1.33	6.05	3.1×
2035	21,544	9,746	64,466	2,20,475	1.39	6.20	3.0×
2036	22,734	10,202	65,664	2,26,540	1.47	6.35	2.9×

Steady Formalisation

YEAR	PSYCHIATRISTS	CLINICAL PSYCH.	COUNSELLORS	WIDER LAYER	PSYCH./LAKH	ALL/LAKH	C : P RATIO
2026	12,942	5,074	54,263	1,70,928	0.89	4.94	4.2×
2027	13,673	5,797	58,365	1,84,726	0.93	5.28	4.3×
2028	14,471	6,545	63,765	2,02,773	0.97	5.71	4.4×
2029	15,342	7,318	70,842	2,26,339	1.03	6.26	4.6×
2030	16,291	8,119	79,728	2,55,927	1.08	6.93	4.9×
2031	17,325	8,950	89,231	2,87,769	1.15	7.64	5.2×
2032	18,449	9,811	97,581	3,16,163	1.21	8.27	5.3×
2033	19,671	10,704	1,04,677	3,40,724	1.29	8.83	5.3×
2034	20,998	11,633	1,10,645	3,61,810	1.37	9.32	5.3×
2035	22,439	12,598	1,16,266	3,81,933	1.45	9.79	5.2×
2036	24,002	13,602	1,21,827	4,02,030	1.55	10.27	5.1×

Demand Unlock

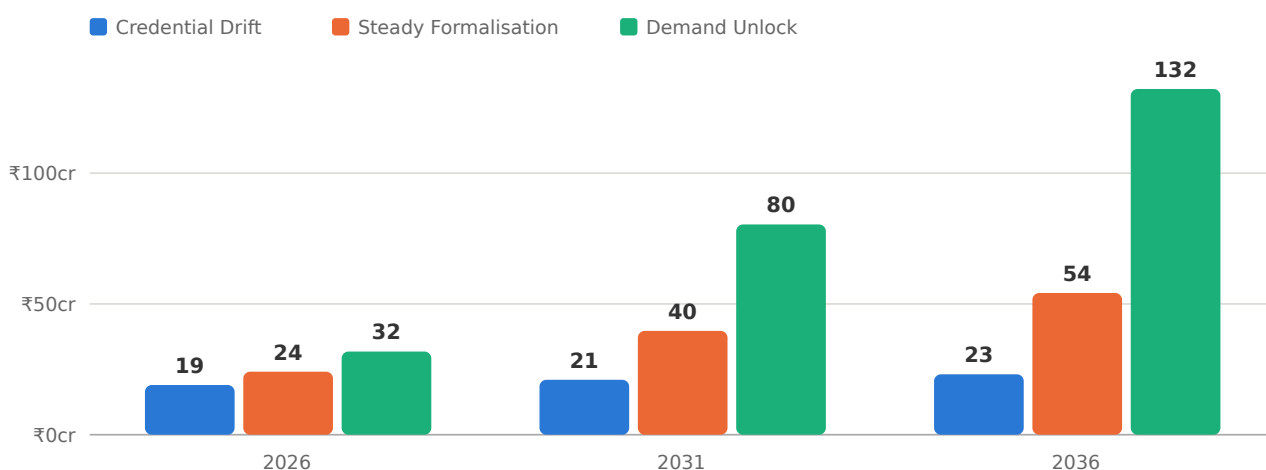
YEAR	PSYCHIATRISTS	CLINICAL PSYCH.	COUNSELLORS	WIDER LAYER	PSYCH./LAKH	ALL/LAKH	C : P RATIO
2026	12,942	5,074	54,263	1,70,928	0.89	4.94	4.2×
2027	13,639	6,062	62,122	1,95,375	0.93	5.55	4.5×
2028	14,433	7,139	73,820	2,31,795	0.97	6.43	5.1×
2029	15,334	8,313	90,518	2,83,774	1.03	7.64	5.9×
2030	16,353	9,594	1,12,710	3,52,784	1.09	9.22	6.9×
2031	17,503	10,994	1,37,159	4,28,621	1.16	10.95	7.8×
2032	18,798	12,523	1,58,940	4,95,893	1.24	12.51	8.5×
2033	20,252	14,197	1,77,731	5,53,634	1.32	13.87	8.8×
2034	21,884	16,028	1,93,863	6,02,913	1.42	15.08	8.9×
2035	23,713	18,033	2,09,524	6,50,572	1.53	16.26	8.8×
2036	25,758	20,230	2,25,575	6,99,283	1.66	17.50	8.8×

The workforce grows. The addressable market stays small.

Aman sells to solo counsellors and never to organisations, so every institution-employed practitioner — hospital, school, NGO, government, EAP panel — leaves the universe entirely rather than being discounted within it. What remains is solo private practice, digitally reachable, self-serve, paying in rupees.

Serviceable market at full penetration

Solo private-practice counsellors × digitally reachable × able to pay, at ₹999 and ₹1,999 per month. This is a ceiling, not a forecast.



For scale: the combined disclosed FY24 revenue of India's ten best-known digital mental health platforms — Wysa, Amaha, YourDOST and seven others — was roughly ₹103 crore. Any market-size claim materially above that is a claim about a market that does not yet transact.

The affordability wall is the finding that matters most. Pro at ₹1,999/month is ₹24,000 a year. A counsellor earning ₹3 lakh gross — the majority, per the 2026 practitioner survey — would be spending **8% of gross revenue** on practice software. That is not a pricing objection to negotiate past; it is a structural ceiling on how much of the workforce can ever be a Pro customer, and it does not move until practitioner earnings move. Which is another way of saying: the thing that grows Aman's market is the same payer event that defines Scenario C.

Three implications follow, and they are uncomfortable in a useful way.

The India-only solo-counsellor SaaS ceiling is low. Under the base scenario, capturing every paying-able solo counsellor in India in 2036 is roughly ₹54 crore of annual revenue. That is a real business and a poor venture story on its own. The investable thesis cannot be “practice management for Indian therapists”; it has to be what the practice data compounds into.

Timing is against a pure wedge play and for the data asset. The admin layer has no pricing power — Zoho gives an equivalent away free, and PractiPal already undercuts at ₹1,499. The scarce thing is longitudinal outcome data across many practices, which is exactly what nobody in India holds and what cannot be bought later. The benchmark pool is the only asset in the roadmap that gets harder to replicate over time.

The 98% is the customer. The 14,759 psychology postgraduates a year who cannot get one of ~290 clinical psychology seats are the population that becomes unregulated counsellors, has no supervisor, no institutional quality signal and no way to know whether they are any good. That is precisely the person for whom private evidence of their own effectiveness is worth paying for. The regulatory gap that makes this market hard to count is the same gap that creates the need.

Honest state, carried into any investor conversation. Aman has zero revenue and one free tenant. There is no payment collection, no trial enforcement, no onboarding and no data import. Automated measure delivery does not exist — counsellors type scores by hand. The Outcome Report does not exist. Pricing is inferred from this analysis, not validated with a single paying customer. Every number in section 05 is a model output, not a result.

What each number is worth

INPUT	VALUE	BASIS
PG psychology out-turn	14,759/yr (2023–24)	AISHE Table 37, primary PDF, independently re-verified HARD
PG psychology enrolment	54,484	AISHE Table 13, primary PDF, re-verified HARD
MSW out-turn	24,645/yr	AISHE Table 37, primary PDF, re-verified HARD
Female LFPR	41.7%	PLFS 2023–24 via Economic Survey 2025–26 HARD
WHO Atlas benchmarks	18 countries	Primary country-profile PDFs; self-reported by governments HARD
Therapist earnings	Majority ≤₹3L/yr	Zensible 2026, n=285, 84% metro — small and metro-skewed FAIR
Psychiatrists 2026	~13,000	Reconstructed here from a 2019 estimate plus seat data FAIR
RCI clinical psychologists	4,309 (Jul 2025)	Parliament reply, but re-verification could not reach the primary FAIR
MD Psychiatry seats	1,292 (2024–25)	NMC seat matrix via secondary report; primary unreachable FAIR
M.Phil clin. psych. seats	~290/yr	Commercial aggregators only; contradicted by register growth WEAK
Practice conversion rate	30%	No India datum exists; anchored on China's 30–40% WEAK
CBSE counsellor mandate	1 per 500 students	Circular not located on CBSE's own index; treat as unconfirmed WEAK
Platform FY24 revenue	~₹103 cr	Aggregator of MCA filings; primary filings not checked WEAK
Wider adjacent layer	~171,000 (2026)	Multiplier assumption; the least defensible figure here WEAK

What would change the answer

NMHS-2 reports on 10 October 2026. India's second National Mental Health Survey has completed fieldwork in 24 states with over 250,000 interviews. Every prevalence and treatment-gap figure in circulation today is from 2015–16. If NMHS-2 shows materially higher prevalence with a similar treatment gap, the demand ceiling re-rates and the binding constraint could flip from demand to supply inside the decade. This model should be rebuilt the week it publishes.

The M.Phil to M.Clin.Psy transition. RCI resolved in April 2026 to replace the M.Phil nomenclature with a Master of Clinical Psychology. Detailed regulations were still pending as of August 2026. If the transition expands capacity, the licensed tier changes; if it merely renames, the 2% licensing bottleneck persists and the informal layer stays the whole story.

NCAHP implementation. All ten professional councils were notified in March 2025, including a Community Care and Behavioural Health council. Whether it builds a real registration pathway for counsellors is the single largest fork in these projections.

Known gaps

AISHE does not disaggregate psychology at undergraduate level — it reports Social Science as one lumped row — so the UG feeder is unquantifiable from official data. No India-specific study exists on how many qualified psychiatrists or psychologists emigrate, leave the field or never practise. Tele-MANAS does not disclose its counsellor headcount. No national DMHP vacancy rate is published. No credible aggregated psychiatrist consultation-fee data exists. Every major teletherapy platform except three declines to publish practitioner counts.

Principal sources

- [AISHE 2023–24 \(14th edition\)](#), Ministry of Education — Tables 13 and 37. The load-bearing source.
- [WHO Mental Health Atlas 2017 — India](#) and the 2020 country profiles for all comparators.
- [Garg, Kumar & Chandra, Indian Journal of Psychiatry 2019](#) — origin of the 0.75/100,000 figure.
- [National Mental Health Survey](#), NIMHANS — 2015–16 results; NMHS-2 pending.
- [Yin et al., scoping review of China's mental health workforce](#) — the certification-boom evidence.
- [US Bureau of Labor Statistics](#) — 483,500 counsellors, +17% projected 2024–34.
- [Zensible, Financial State of Therapy Practice in India 2026](#) — practitioner earnings.
- [Economic Survey 2025–26, ch. 12](#) — PLFS labour force participation.
- [CMHLP India Mental Health Observatory Budget Brief 2026](#) — DMHP and Tele-MANAS funding.

Model, assumptions and scenario parameters are reproducible; every parameter is documented inline in the source model. Figures marked Fair or Weak above should not be quoted without the qualifier attached.